

THE ACCUSATION REVIEW · 2026

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# What 32 Aesthetic Board Accusations Reveal

The patterns, outcomes and costs behind 32 licensing Board accusations against nurses, NPs, PAs and physicians in aesthetics, and the documents that would have made the difference.

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22 of 32

cited missing or insufficient  
standardized procedures

20 of 32

ended in 3 years'  
license probation

\$7,592

median cost recovery  
owed to the Board

## WHY I DID THIS

# Most Board cases aren't decided by what happened in the chair.

In more than 18 years in aesthetic nursing, I have helped providers facing possible licensing issues. Along the way, I kept hearing the same question: What does the Board actually look for?

So I read 32 licensing Board accusations filed against registered nurses, nurse practitioners, physician assistants and physicians working in aesthetics. I charted every citation, every outcome and every dollar of cost recovery. This brief is what I found.

The short version: only 7 of the 32 cases involved documented patient harm. Most began with a complaint, a tip, a customs seizure or an undercover appointment, and they were decided on documentation: standardized procedures, good faith exams and physician oversight.

## AT A GLANCE

32

accusations reviewed across RN, NP, PA and physician licenses

22

cited missing or insufficient standardized procedures or P&Ps

7

involved documented patient harm

20

ended in 3 years' probation (revoked, stayed); one more got 35 months

\$7,592

median cost recovery owed to the Board (20 cases with an amount)

~3 yrs

typical gap between the conduct and the filed accusation

## WHO WAS CHARGED

|                          |    |
|--------------------------|----|
| Registered nurses        | 21 |
| RN / nurse practitioners | 2  |
| Nurse practitioners      | 1  |
| Physicians               | 6  |
| Physician assistants     | 2  |

FINDING 01

# The same gaps, again and again

How often each problem appeared across the 32 accusations. A case is counted if the issue appears anywhere in its description or citations, so one case can appear in several rows.

|                                                                   |          |
|-------------------------------------------------------------------|----------|
| No or insufficient standardized procedures / P&Ps                 | 22 of 32 |
| Inadequate physician oversight or supervision                     | 19 of 32 |
| No good faith exam or medical assessment before treatment         | 17 of 32 |
| Undercover agent posed as a patient                               | 12 of 32 |
| Improper corporate structure, ownership or fictitious name permit | 8 of 32  |
| Documentation failures: charts, consent, lot numbers, signatures  | 8 of 32  |
| Patient harm: burns, infection, suspected occlusion, paralysis    | 7 of 32  |
| Arrest or criminal conviction for unlicensed practice             | 6 of 32  |
| Non-FDA-approved or foreign-sourced product                       | 5 of 32  |
| Tasks delegated to someone without the license for them           | 4 of 32  |

Paperwork decided most cases.

The top three findings are all documentation findings. They exist, or they don't, long before anyone asks.

Generic policies failed.

One nurse submitted a national association's policy set; the Board found it did not meet standardized procedure criteria. Another practice tried to update its P&Ps mid-investigation and still fell short.

Oversight must be real.

An out-of-state medical director covering four medspas. Charts never signed. A medical director with no training in the treatments he oversaw.

The exam must be done by the right person.

Exams by a third party, remote exams with no plan or discussion, or none at all.

## FINDING 02

# What it costs

An accusation is not the end of the story; it is the start of years of consequences. Here is how the 32 cases resolved.



Each segment = share of 32 cases

|                        |    |                                                                |
|------------------------|----|----------------------------------------------------------------|
| 3-year probation       | 20 | License revoked, with revocation stayed and 3 years' probation |
| 35-month probation     | 1  | A physician case                                               |
| Public reproof         | 5  | A permanent, public disciplinary record                        |
| Surrendered or revoked | 4  | Three surrenders, one revocation and suspension                |
| Pending                | 2  | Still open at the time of review                               |

## COST RECOVERY OWED TO THE BOARD



Cost recovery reimburses the Board for its investigation and prosecution. It does not include attorney fees, probation monitoring, required coursework, lost shifts, or the cost of rebuilding a practice's systems.

## TIME TO ACCUSATION

Across 29 cases with dates, the typical gap between the conduct and the filed accusation was about three years; 19 took three years or longer, and the longest took five. By then, staff, owners and medical directors had often changed. The records were what remained.

FINDING 03

# Six cases, six lessons

Anonymized and simplified. Each case turns on its own facts.

CASE A · DERMAL FILLER · RN

"The Board asked for her policies. There were none to send."

|                      |                                                                                                                              |
|----------------------|------------------------------------------------------------------------------------------------------------------------------|
| WHAT HAPPENED        | A patient complained about dermal filler she said she did not want. The Board requested the nurse's policies and procedures. |
| WHAT THE BOARD FOUND | None could be provided. The accusation charged incompetence.                                                                 |
| OUTCOME              | Pending final decision                                                                                                       |

THE DOCUMENT THAT WOULD HAVE HELPED

Standardized procedures for dermal filler, a written complaint-response protocol, and a signed informed consent specific to the treatment.

CASE B · KYBELLA · RN

"She had policies. They weren't standardized procedures."

|                      |                                                                                                                                                                                                                                               |
|----------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| WHAT HAPPENED        | A patient developed a serious infection after Kybella and reported it to the Board.                                                                                                                                                           |
| WHAT THE BOARD FOUND | Gross negligence and incompetence. The nurse had provided a national association's policy set, which did not meet standardized procedure criteria. The Board also cited missing physician follow-up and an incorrect lot number in the chart. |
| OUTCOME              | Revoked, stayed, 3 years' probation; \$14,728 cost recovery                                                                                                                                                                                   |

THE DOCUMENT THAT WOULD HAVE HELPED

State-specific standardized procedures approved by the medical director, an adverse-event and physician follow-up protocol, and a lot-tracking log reconciled to the chart.

CASE C · LASER HAIR REMOVAL · RN

"One skipped test spot."

|                      |                                                                                                                     |
|----------------------|---------------------------------------------------------------------------------------------------------------------|
| WHAT HAPPENED        | A good faith exam was completed. Three months later, laser hair removal at an incorrect setting burned the patient. |
| WHAT THE BOARD FOUND | Incompetence and unprofessional conduct for failing to perform a test spot.                                         |
| OUTCOME              | Revoked, stayed, 3 years' probation; \$10,000 cost recovery                                                         |

THE DOCUMENT THAT WOULD HAVE HELPED

A laser standardized procedure that requires and documents a test spot, a device-settings log, and a signed provider competency for the device.

CASE D · LASER · OUT-OF-STATE MEDICAL DIRECTOR

## "A name on a contract is not oversight."

|                      |                                                                                                                                                                |
|----------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------|
| WHAT HAPPENED        | The medical director lived in another state and oversaw four medspas. Nurses performed laser treatments without physician orders, supervision or exams.        |
| WHAT THE BOARD FOUND | Gross negligence, practicing medicine without a license, and aiding and abetting, because the nurse managed staff while knowing supervision was not happening. |
| OUTCOME              | Public reproof                                                                                                                                                 |

### THE DOCUMENT THAT WOULD HAVE HELPED

A medical director agreement with defined supervision duties, a chart-review log, and good faith exam records for every patient.

CASE E · NEUROTOXIN AND FILLER · RN

## "It started with a package at customs."

|                      |                                                                                                                                                                  |
|----------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| WHAT HAPPENED        | The FDA intercepted fillers and neurotoxin shipped from overseas to a nurse's home. An undercover agent then booked an appointment; the exam was done remotely.  |
| WHAT THE BOARD FOUND | Unprofessional conduct and incompetence for obtaining non-FDA-approved products, advertising medical treatments, and practicing without standardized procedures. |
| OUTCOME              | Revoked, stayed, 3 years' probation; \$18,688 cost recovery                                                                                                      |

### THE DOCUMENT THAT WOULD HAVE HELPED

A product sourcing and purchasing policy, a good faith exam policy, and an advertising review policy.

CASE F · TWO NURSES, ONE PRACTICE · RN

## "The update came too late."

|                      |                                                                                                                                                                                        |
|----------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| WHAT HAPPENED        | After a complaint about oversight, the Board sent an undercover agent. An RN assessed and confirmed the treatment plan; no physician was involved. Agents seized product.              |
| WHAT THE BOARD FOUND | The practice was not a medical corporation and had no fictitious name permit. P&Ps were not in place, and an attempt to update them failed to meet criteria. Both nurses were charged. |
| OUTCOME              | Both: revoked, stayed, 3 years' probation; \$5,571 and \$6,726 cost recovery                                                                                                           |

### THE DOCUMENT THAT WOULD HAVE HELPED

A corporate structure review before opening, and standardized procedures that meet state criteria from day one.

## SELF-AUDIT

# 15 questions to ask this week

Every item below maps to a finding in the 32 accusations. Any item you can't check with confidence is worth a closer look.

- ☐ 01 Written standardized procedures exist for every treatment we offer, meet our state's criteria, and are signed by the medical director.
- ☐ 02 Our P&Ps were written for our state and scope, not adapted from a generic or association template.
- ☐ 03 Every patient has a documented good faith exam by a physician, NP or PA before treatment begins.
- ☐ 04 Remote or telehealth exams include a documented plan and communication with the treating nurse.
- ☐ 05 Our medical director agreement defines supervision duties, and chart reviews are logged.
- ☐ 06 Our medical director is trained in, and available for, the treatments being supervised.
- ☐ 07 Every chart includes medical history, treatment-specific consent, product, dose, and correct lot number.
- ☐ 08 Laser and energy device protocols require and document test spots and settings.
- ☐ 09 Every provider has signed competencies for each treatment they perform.
- ☐ 10 Adverse events trigger a written protocol, including physician follow-up and documentation.
- ☐ 11 All products are purchased through a legitimate U.S. supply chain, under our own license.
- ☐ 12 Our corporate structure, ownership and fictitious name permit meet our state's rules.
- ☐ 13 No task is delegated to anyone without the license to perform it (estheticians, LVNs, assistants).
- ☐ 14 Our website and social media claims have been reviewed for advertising compliance.
- ☐ 15 If the Board requested our P&Ps today, we could send them within 24 hours.

Prefer to answer online? Take the 2-minute readiness self-check at [aestheticnursingpnp.com](https://aestheticnursingpnp.com)

## NEXT STEPS

# Fix it before anyone knocks.

My Policies and Procedures have been reviewed by State Boards as part of real Board matters. Here is how I can help, wherever you stand.

|                                   |              |                                                                                                                                                                                                         |
|-----------------------------------|--------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Compliance Readiness Audit        | \$950        | Pre-audit document review, a 90-minute walkthrough with the owner and medical director, and a scored gap report. The fee is credited toward a Custom Manual booked within 60 days.                      |
| Custom State-Specific Manual      | From \$1,800 | Standardized procedures, consent forms, charting templates, patient instructions and provider competencies for each treatment, with two team training sessions.                                         |
| Investigation & Probation Support | By consult   | For nurses and providers facing a Board complaint, investigation or accusation, or on probation. Coordinated with your attorney.                                                                        |
| Continuing Education              | From \$40    | From Complaint to Accusation (2 CE), Good Faith Exam (1 CE), Standardized Procedures (1 CE), and Guidelines for a Compliant Practice: Pillars of Success (3 CE).<br>CA BRN-Approved CE Provider #18009. |

### BOOK A CONSULT

Cindi Vokey, BSN, RN

[aestheticnursingpnp.com](http://aestheticnursingpnp.com)

[info@aestheticnursingpnp.com](mailto:info@aestheticnursingpnp.com) · 508-498-6604

Courses: [aestheticnursingces.com](http://aestheticnursingces.com)

About this review. Summaries are drawn from 32 publicly filed licensing Board accusations reviewed by Cindi Vokey, BSN, RN, and are anonymized and simplified. Counts reflect whether an issue appears anywhere in a case's description or citations; a case can appear in several categories. Cost figures reflect the cost recovery stated in each accusation. Each case turns on its own facts. This brief is education, not legal advice; consult a healthcare attorney about your situation.